

Cardiac Recovery Protocol

Post-Discharge Care for Cardiac Patients | City General Hospital

1. Overview

This protocol guides patients discharged following a cardiac event — including myocardial infarction (MI), heart failure, CABG, or PCI. Adherence to these instructions reduces re-hospitalisation risk and improves long-term outcomes.

The heart needs 4–8 weeks to heal. Gradual return to activity, strict medication compliance, and daily self-monitoring are the three pillars of safe cardiac recovery.

2. Activity Restrictions

- Rest for 24–48 hours post-discharge; limit to short indoor walks only.
- Do not lift objects above 2 kg for the first 2 weeks without medical clearance.
- Do not drive for at least 4 weeks after MI or cardiac surgery.
- Walk 5–10 minutes daily, increasing by 5 minutes per week as tolerated.
- Avoid stairs more than once per hour in week 1; use a single floor if possible.

3. Medication Reminders

Never stop cardiac medications without consulting your doctor. Stopping antiplatelet therapy (aspirin/clopidogrel) risks stent thrombosis within 24 hours.

- Antiplatelet drugs: take with food; never miss a dose.
- Beta-blockers: do not stop suddenly — always taper under medical supervision.
- ACE inhibitors/ARBs: report dry cough or ankle swelling to your doctor promptly.
- Statins: take in the evening; report any muscle pain or weakness immediately.
- Diuretics: weigh daily before eating; alert your team if weight rises > 1 kg/day.

4. Dietary and Lifestyle Guidelines

A heart-healthy diet lowers the risk of further cardiac events. These guidelines align with the Mediterranean diet, the strongest evidence-based dietary pattern for cardiac protection.

- Limit sodium to < 2 g/day; avoid adding salt at the table or during cooking.
- Reduce saturated fat (red meat, butter) and eliminate all trans fats.
- Eat oily fish (salmon, mackerel) at least twice weekly for omega-3 fatty acids.
- Eat 5+ portions of fruit and vegetables daily.
- Limit alcohol to ≤ 1 unit/day; avoid binge drinking during recovery.
- Smoking cessation is mandatory — smoking doubles the risk of a second MI.

5. Blood Pressure and Heart Rate Monitoring

- Measure BP twice daily — before morning medications and before evening meal.
- Sit quietly 5 minutes before measuring; avoid caffeine or exercise for 30 min prior.
- Target BP: < 130/80 mmHg; target resting HR: 55–70 bpm on beta-blocker therapy.
- Contact your doctor if systolic BP > 160 or < 90 on two consecutive readings.

6. Warning Signs — Seek Emergency Care Immediately

■ Call 112 immediately for any of the following. Do not wait. Do not drive yourself to hospital.

- Chest pain, tightness, or pressure lasting more than 5 minutes.
- Pain radiating to the left arm, jaw, neck, or upper back.
- Sudden severe breathlessness at rest or waking unable to breathe.
- Palpitations with dizziness, near-fainting, or loss of consciousness.
- Sudden severe headache, facial droop, or one-sided weakness (stroke signs).
- Weight gain > 2 kg within 48 hours — sign of fluid retention.

7. Wound Care (Surgical Patients)

- Keep the sternal wound dry for 7 days; no submerging in water.
- Sternal precautions for 6 weeks: no lifting > 2 kg, no reaching above shoulder level.
- Report redness, discharge, unusual pain, or sternal clicking to your surgical team.
- Leg graft sites: elevate when seated; wear compression stockings for 4 weeks.

8. Follow-Up Schedule

Missing follow-up appointments is associated with a 40% increase in re-hospitalisation risk. Attend all appointments listed below.

Time Point	Review Type	What to Bring
Day 1	Nurse phone check	Symptom diary
Day 3	GP review	Discharge letter, medication list
Day 7	Wound check + blood tests	BP diary, all medications
Week 4	Cardiologist review	BP/HR diary, exercise log
Week 6	Cardiac rehab assessment	Referral letter
3 Months	Repeat echocardiogram	Current medication list

9. Daily Patient Checklist

- ■ All medications taken at correct time and dose.
- ■ Morning BP and HR recorded in diary.
- ■ Daily weight recorded (same time, same scales, before eating).
- ■ Planned walking exercise completed.
- ■ Low-sodium, heart-healthy diet followed all day.

Cardiology Nurse Helpline for non-urgent queries. Call 112 for emergencies. Synthetic document — for demonstration purposes only.